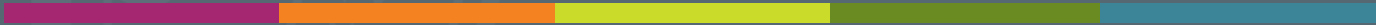


Transforming Certification for Better Care

ABMS CONFERENCE 2026

**SHAPING THE FUTURE OF
RESIDENCY EDUCATION:
Opportunities to Redefine Assessment
and Competency Evaluation**

From Vision to Practice: The American Board of Pediatric's Competency Based Medical Education (CBME) Journey



Brenda Nuncio, MS

Program Coordinator, Competency-Based Medical Education

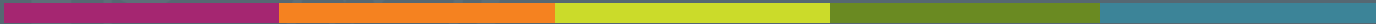
American Board of Pediatrics

Member Boards Connect Session

ABMS Conference 2026

Rosemont, Illinois

No disclosures



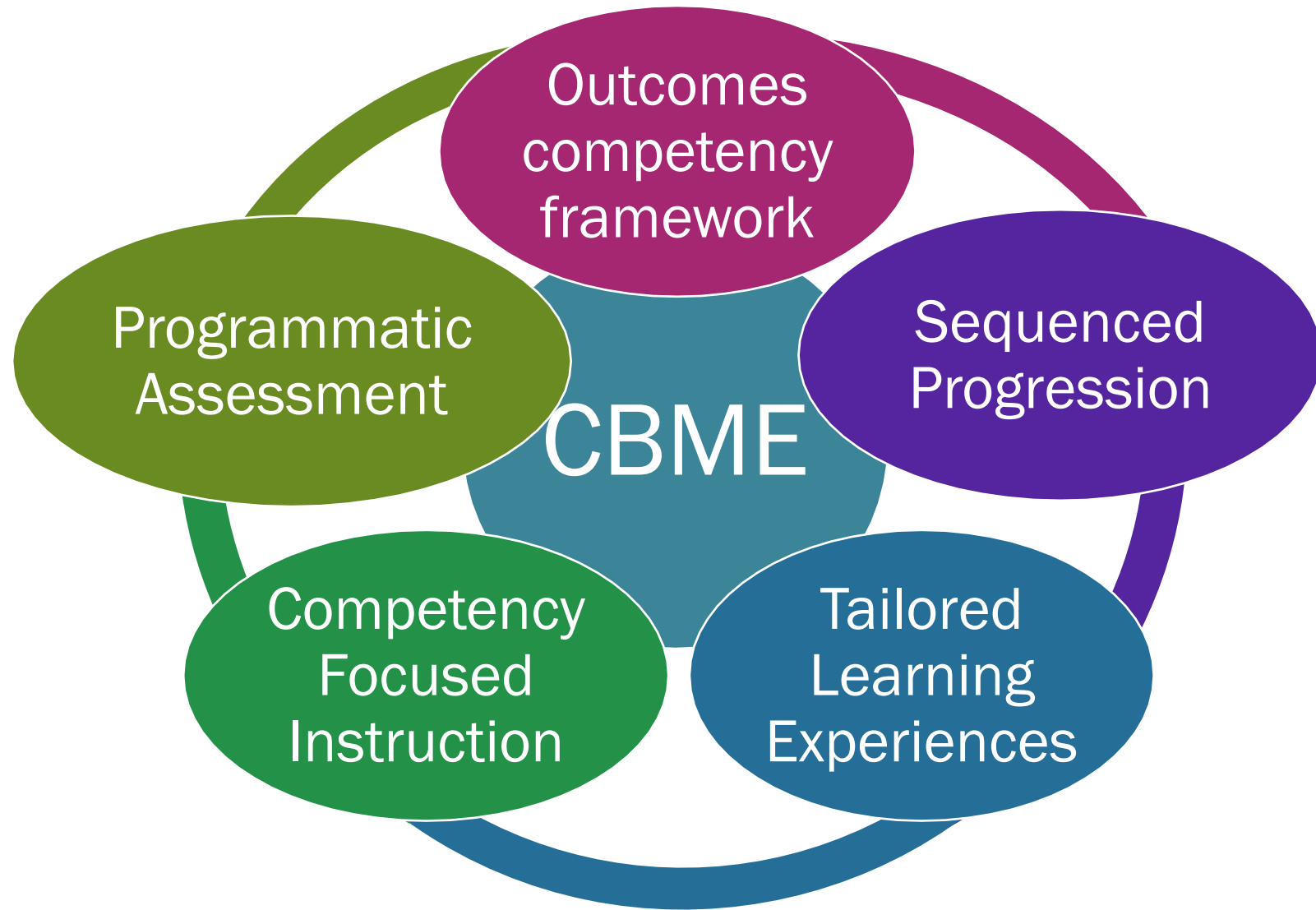


**CBME Is Patient Focused and
Learner Centered**



What is CBME?

CBME is an organizing framework for ***both*** training and certification



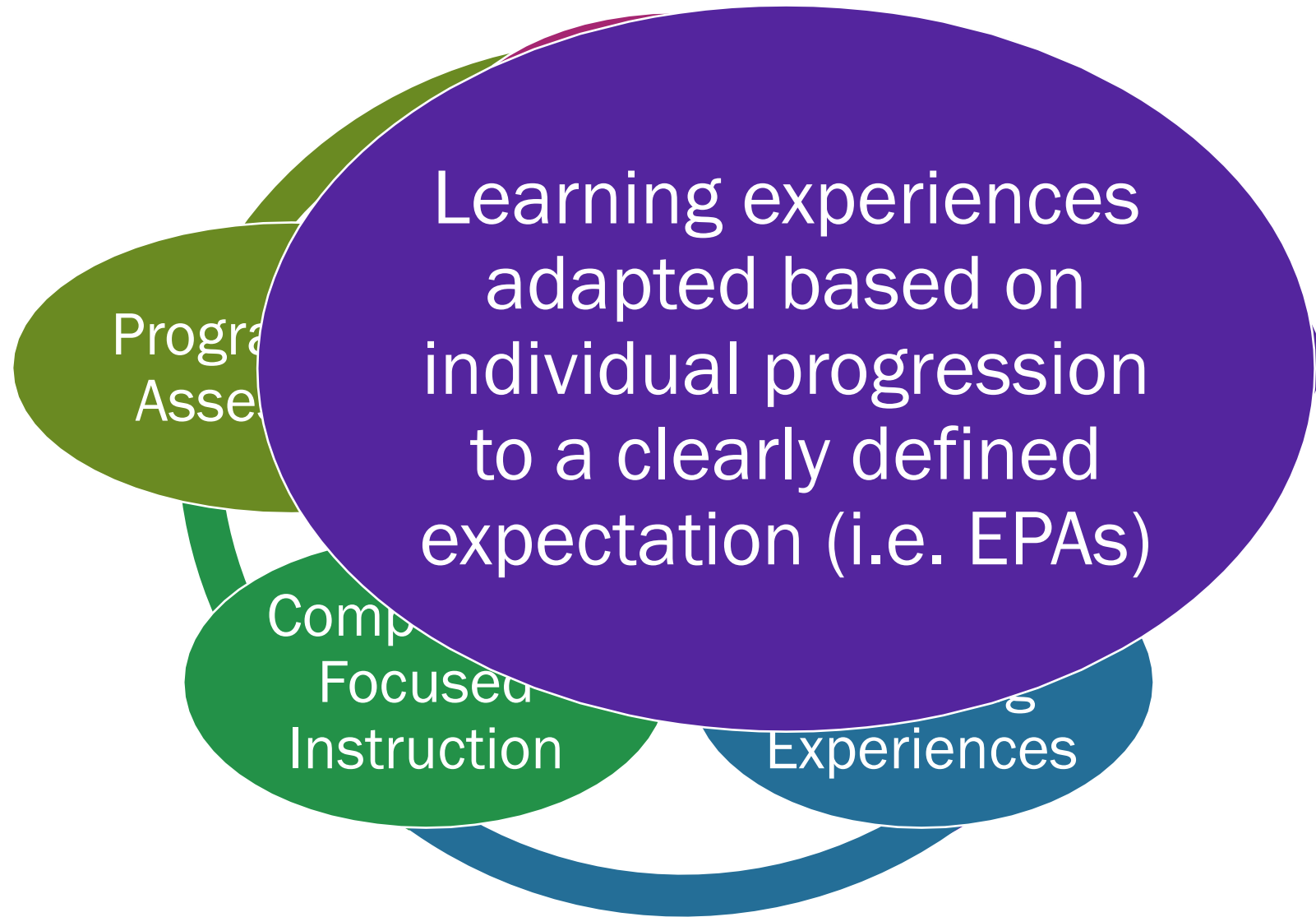
CBME :
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Defining the Outcomes
*Our community came
together to define our
outcomes using the EPA
framework*

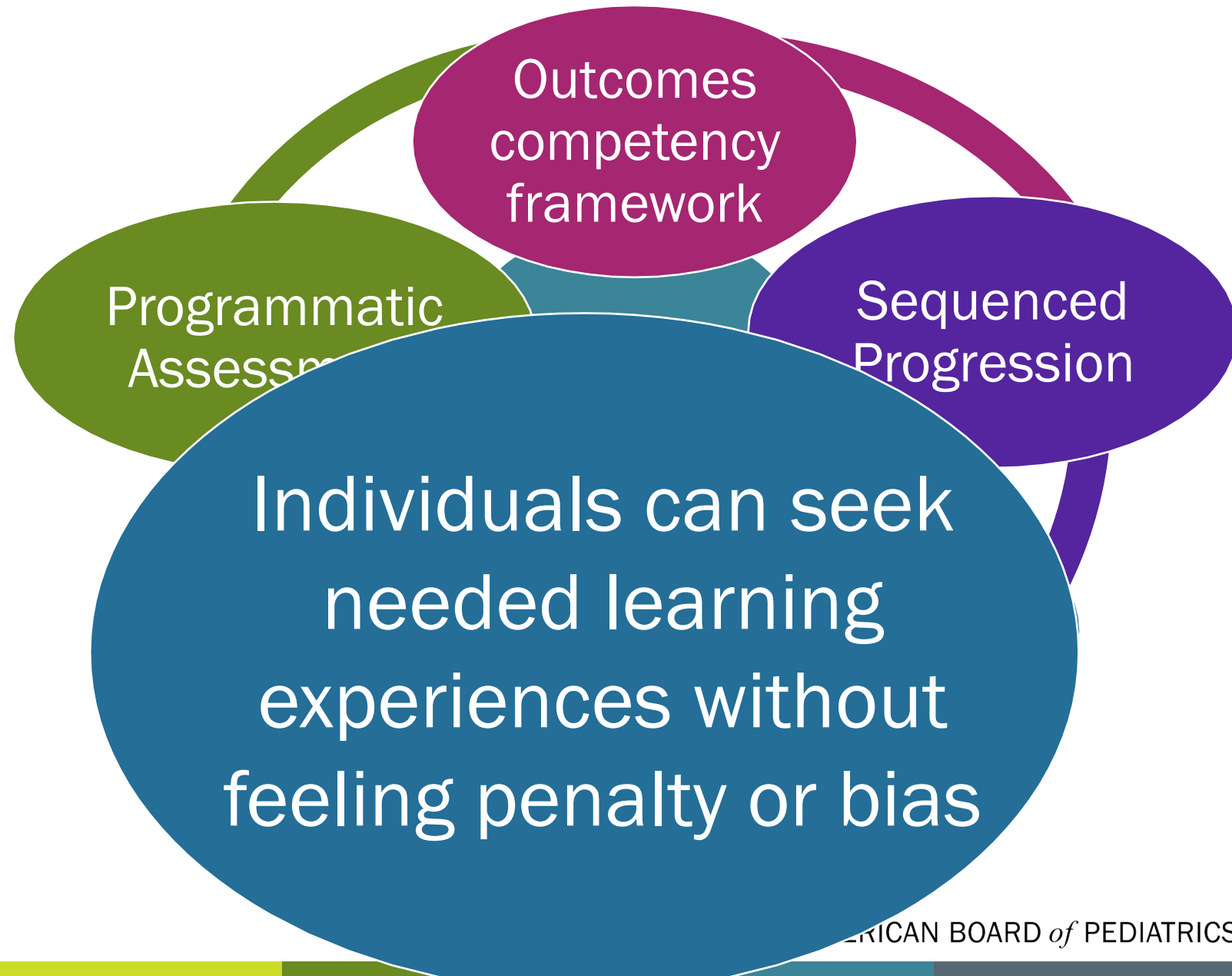
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CBME is an organizing framework for ***both*** training and certification



CBME is an organizing framework for *both* training and certification



CBME is an
organizing
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certification

Teachers tailor their
approach to the
needs of each
learner, based on
clear expectations

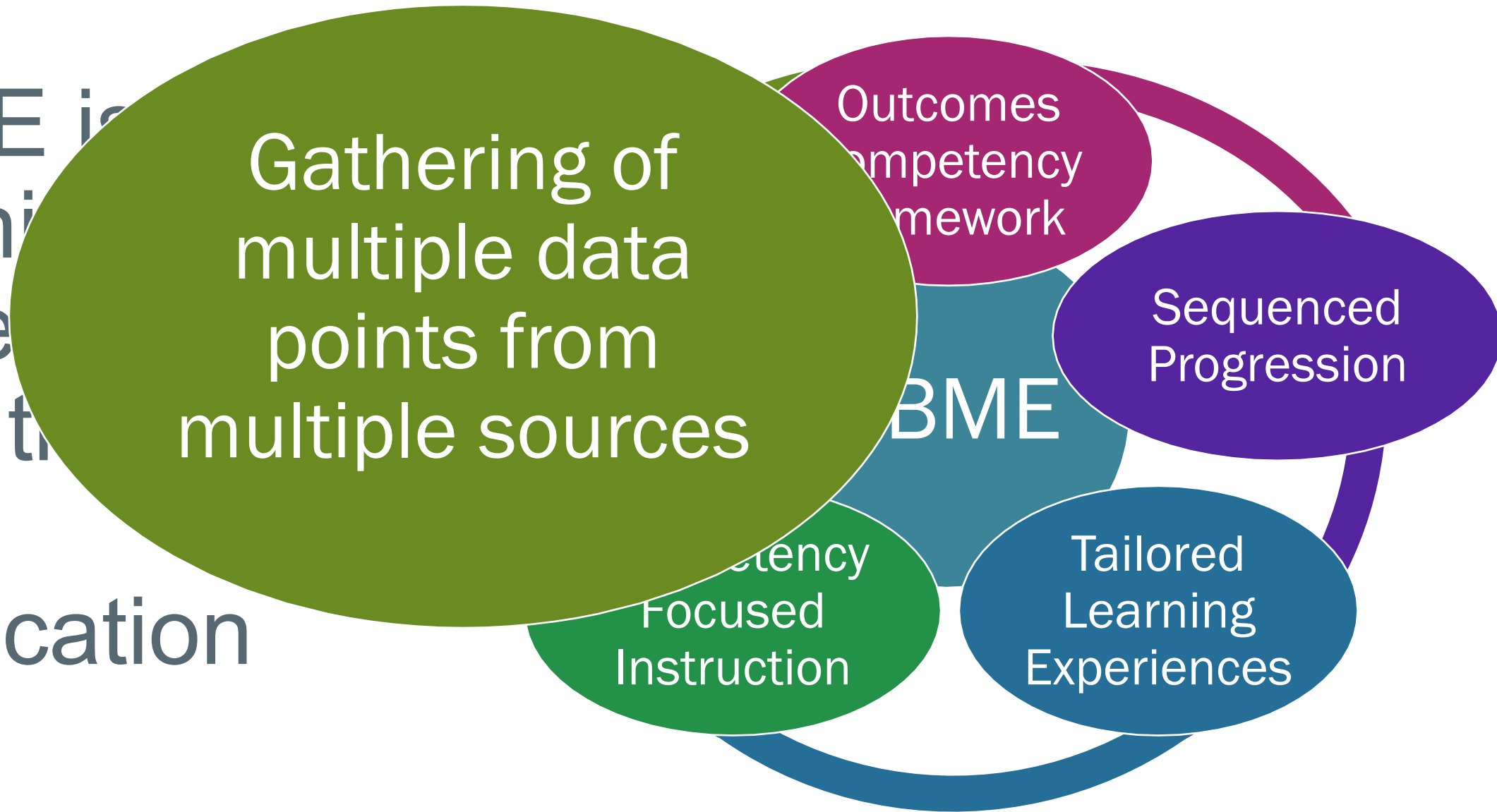
Outcomes
competency
framework

Sequenced
Progression

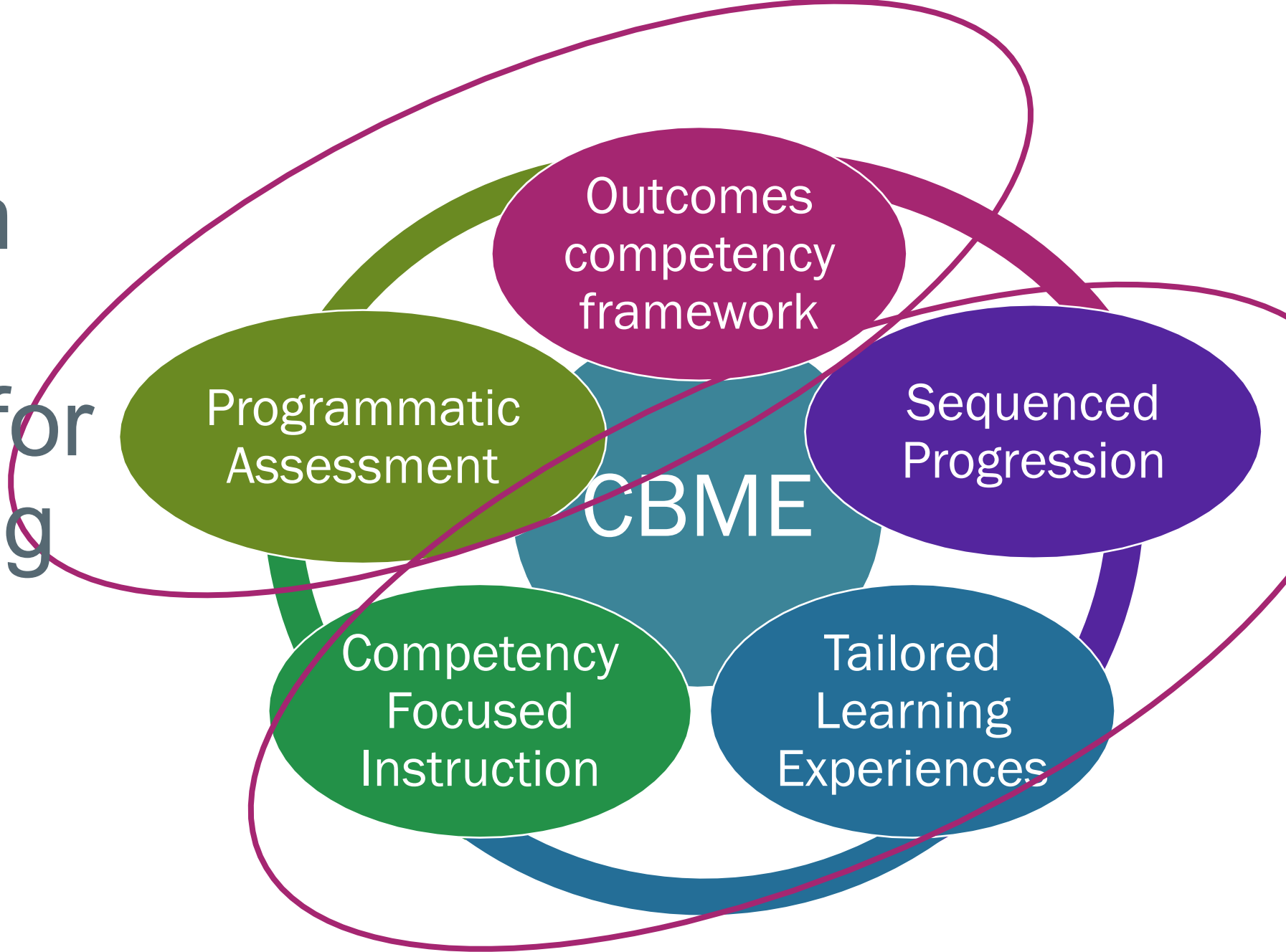
Tailored
Learning
Experiences

CBME

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CBME is an organizing framework for ***both*** training and certification



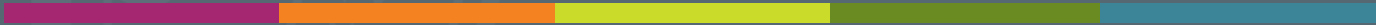
The background is a collage of overlapping rectangular and square pieces in shades of blue and white. A single, solid orange rectangle is positioned vertically in the upper-left quadrant. The text 'Group Activity' is written in a white, serif font, centered horizontally and positioned to the right of the orange rectangle.

Group Activity



Milestones Leave Gaps in Assessment and
Create an Incomplete Picture

From Vision to Practice: The American Board of Pediatrics' Competency Based Medical Education (CBME) Journey



Brenda Nuncio, MS

Program Coordinator, Competency-Based Medical Education

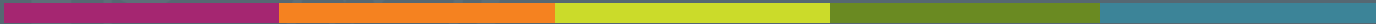
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No disclosures



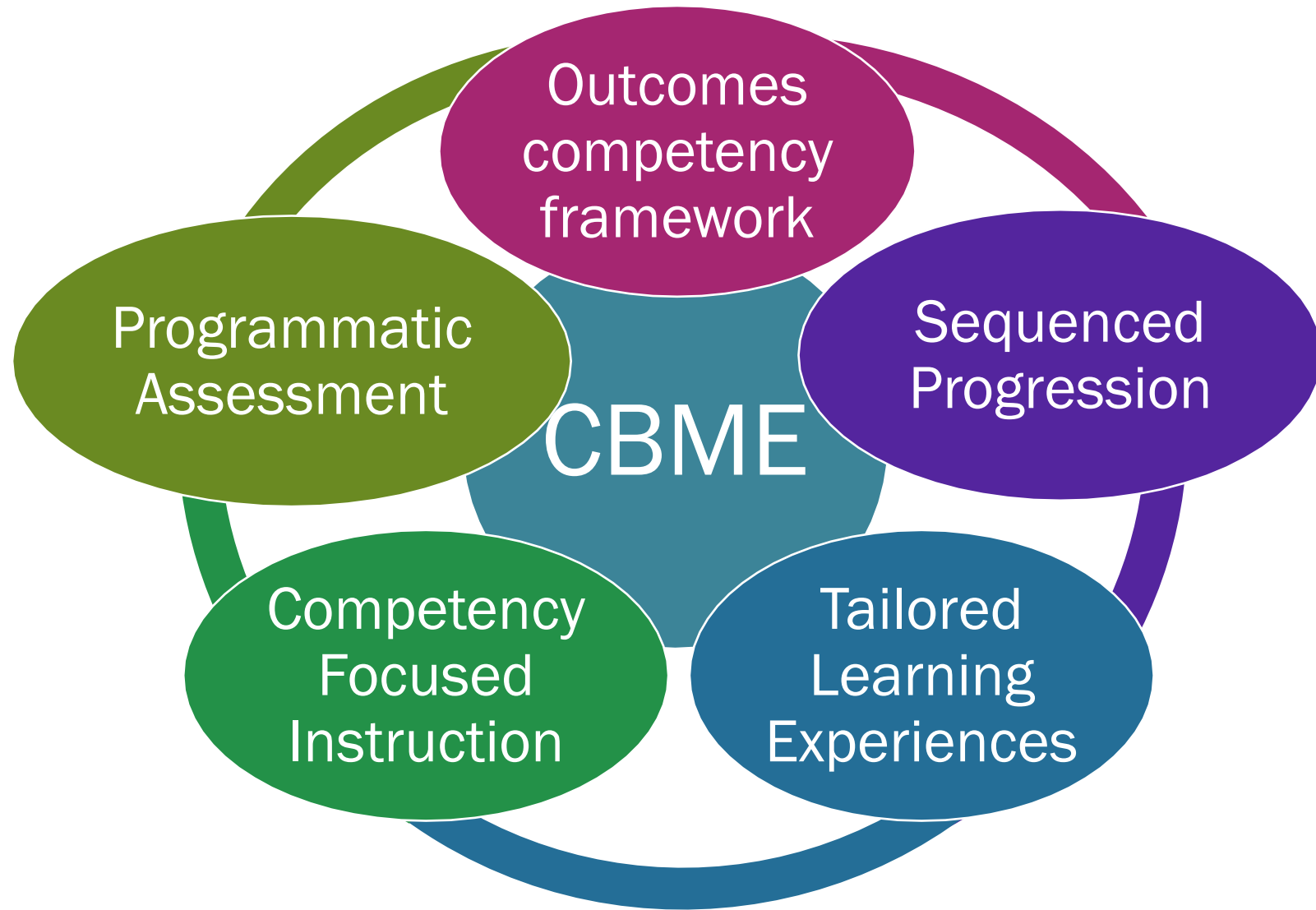


**CBME Is Patient Focused and
Learner Centered**



What is CBME?

CBME is an organizing framework for *both* training and certification



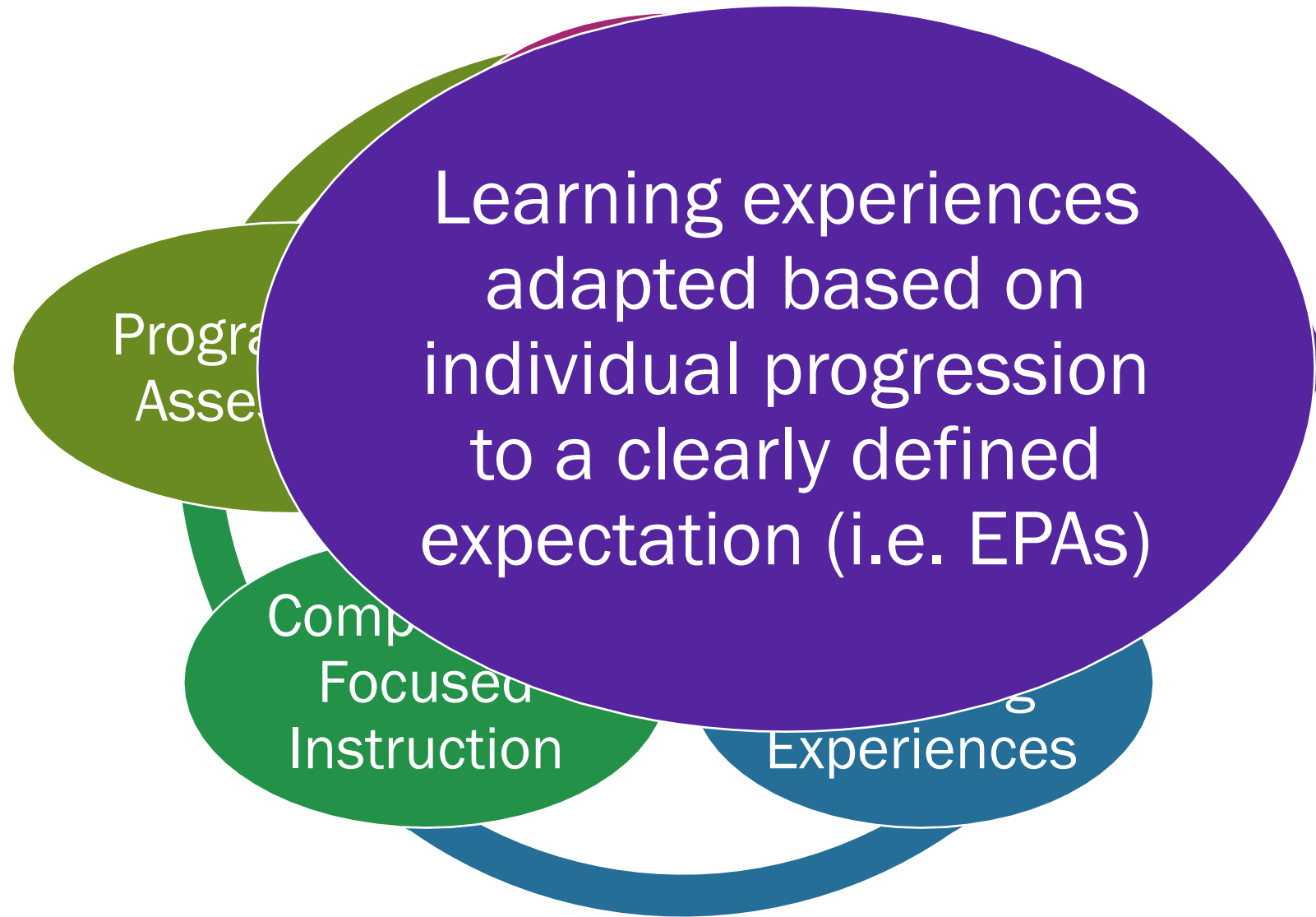
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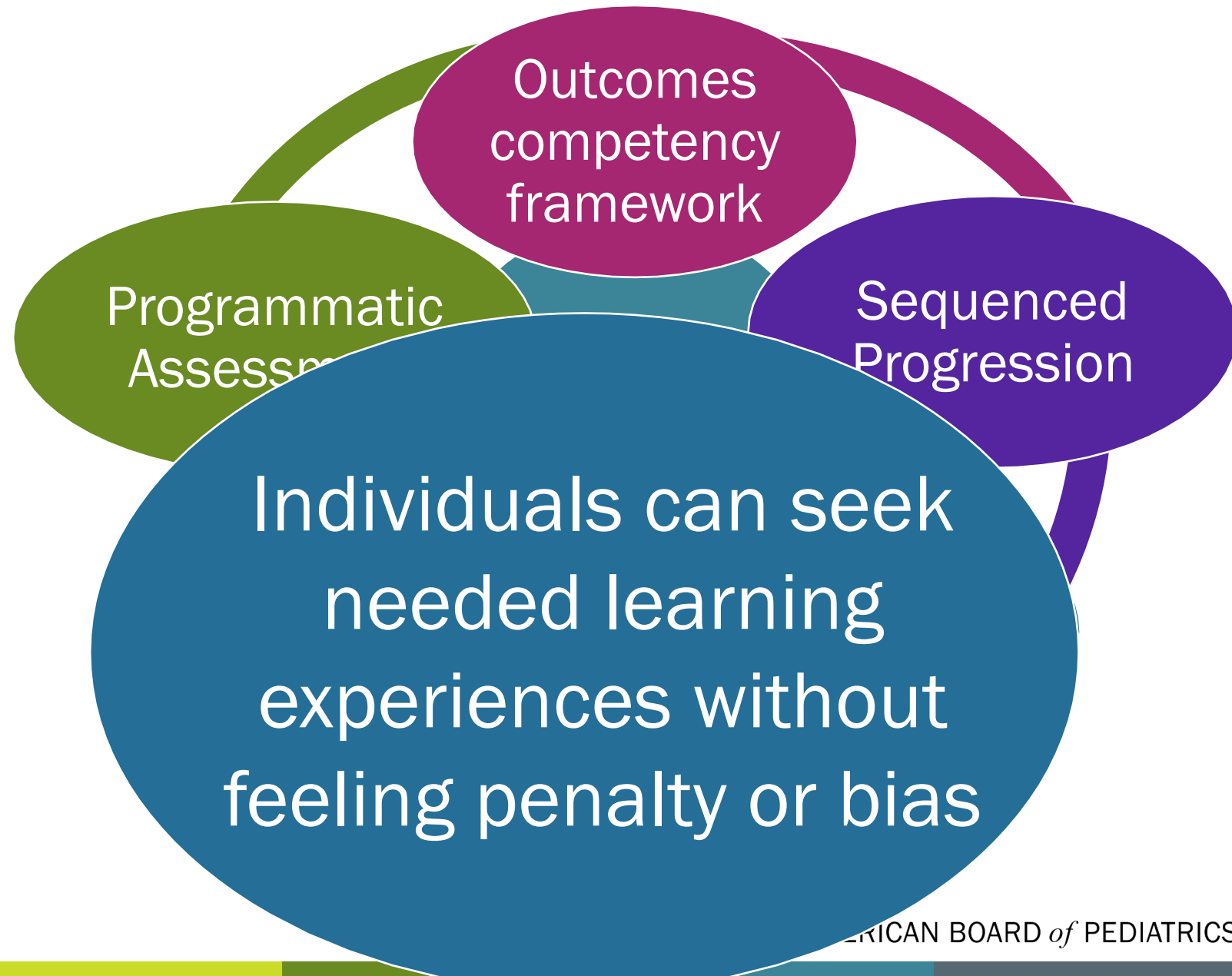
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CBME is an organizing framework for ***both*** training and certification



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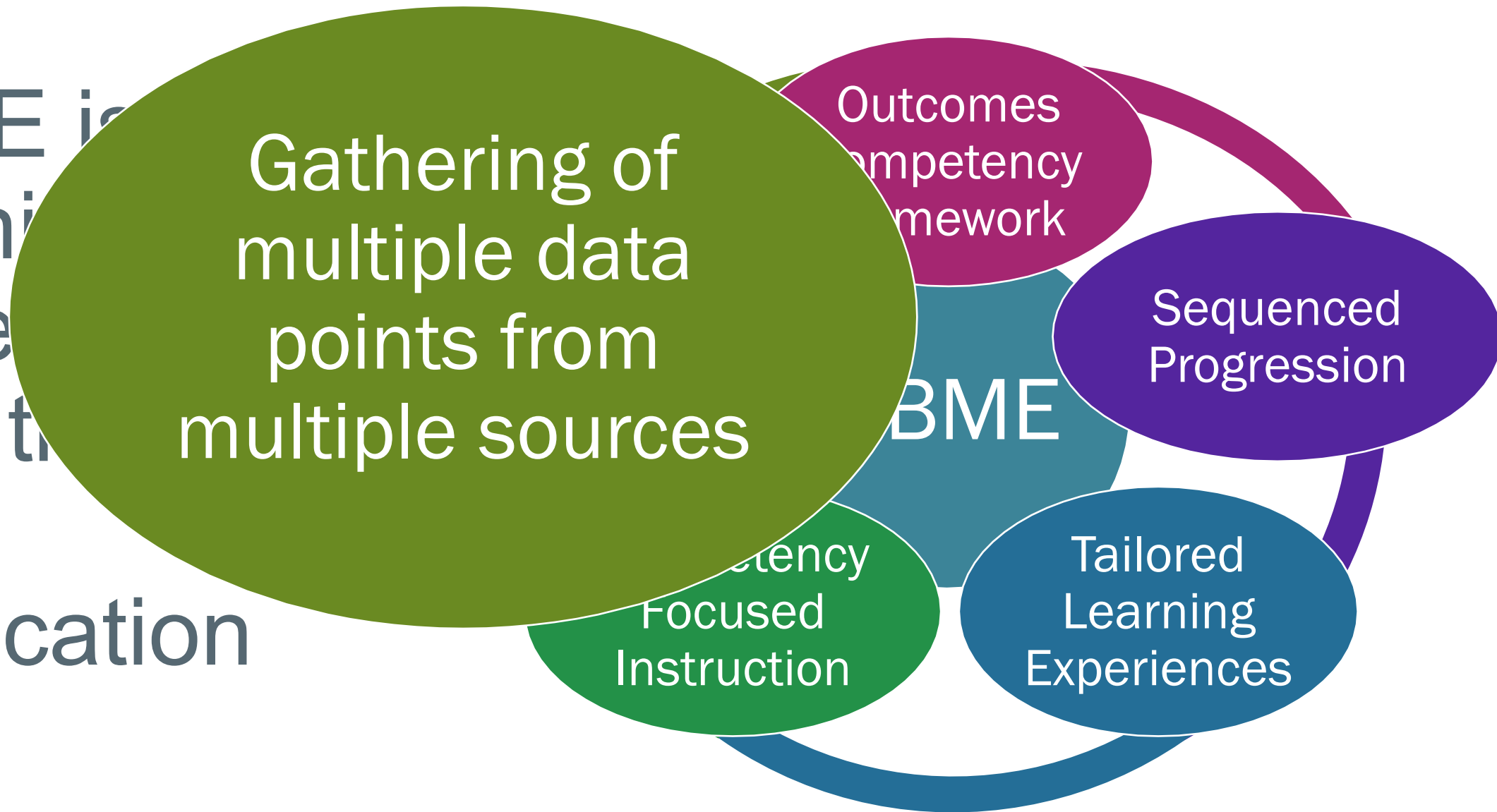
Outcomes
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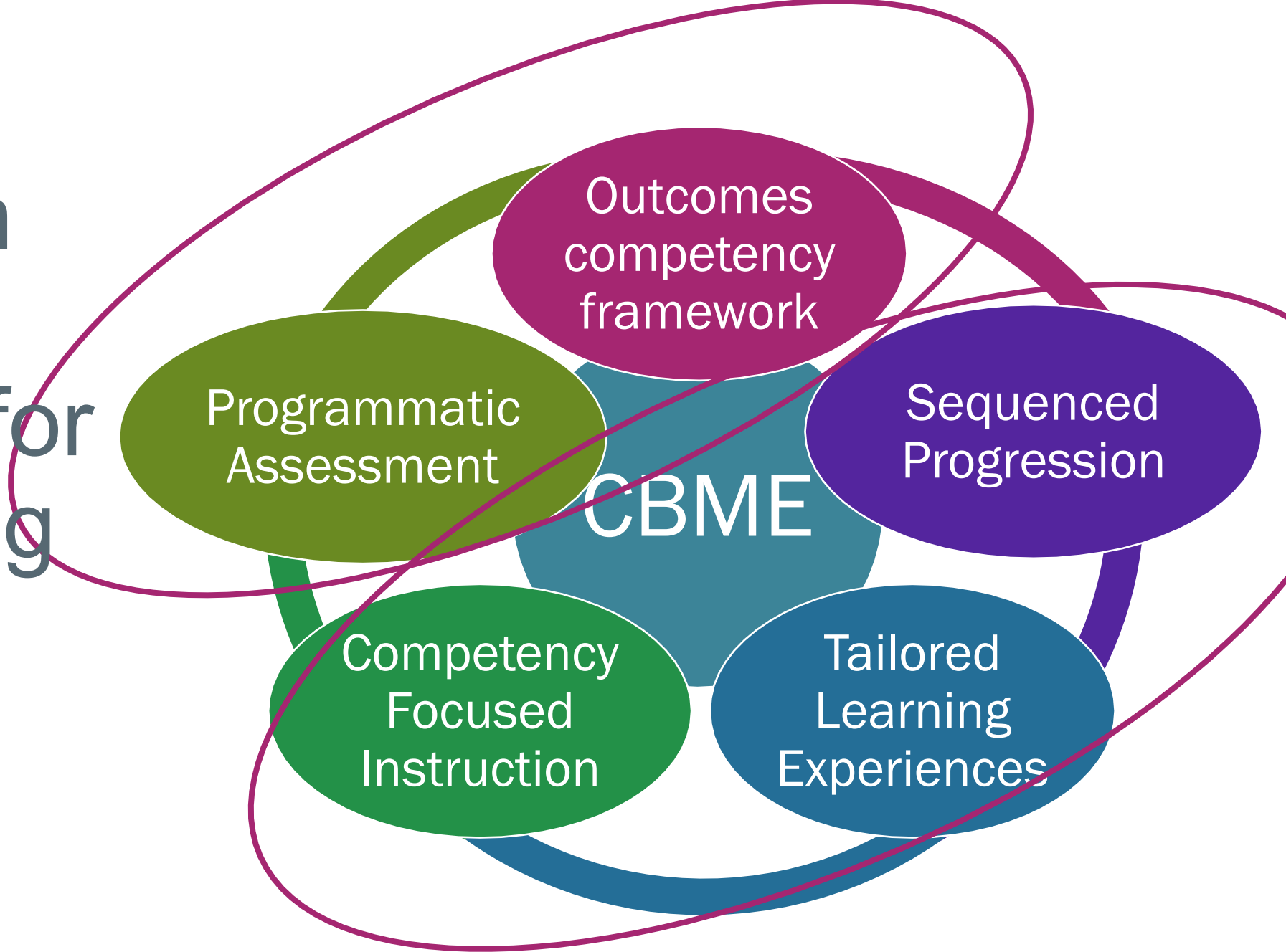
Tailored
Learning
Experiences

CBME

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CBME is an organizing framework for ***both*** training and certification



The background is a collage of overlapping rectangular papers in light blue and white. A single, solid orange rectangle is placed vertically in the center-left area. The text 'Group Activity' is written in a white, serif font, positioned to the right of the orange rectangle.

Group Activity

Stages of Competency Based Medical Education (CBME) Development Within a Specialty

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	• MB representative(s) • ACGME Review Committee leaders • Education leaders within your specialty • Program directors • Department chairs • Experts from other specialties engaged in CBME efforts • Patients and families	• Convene a working group to explore • Identify current educational gaps in your specialty (i.e., readiness for practice of graduating trainees) • Explore existing and/or begin development of clear outcomes in your specialty that are important to patients and families and expected of graduating trainees • Review the assessment practices and current time/volume requirements for certification in your specialty (Historical; Evidence from literature; Patient and public needs)	• Resistance to change within the specialty • Data on educational outcomes for current and future processes not readily available • Unclear about expectations from patients and the public	12 mos
Before next stage: Review data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME				
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	• In addition to above, educational assessment experts in your specialty and from other specialties • Will likely need multiple workgroups	• Communicate current educational and practice gaps for your specialty that are described above • Finalize definitions of expected educational outcomes (e.g., entrustable professional activities (EPAs)) • Identify faculty development needs and materials • Identify technological solutions for implementation • Develop or assess the assessment tools for implementation by GME programs, MB, or both • Determine how certification process might need to evolve to integrate CBME principles and assessment	• Program and faculty awareness • Technological barriers to data collection by programs • Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
Before next stage: Create clearly defined outcomes competencies expected of practicing physicians in the specialty, develop a detailed plan for pilot testing in a few sites that can be expanded to more sites as your specialty learns about what works, for whom, and in what circumstances				
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	• As above • Enlist innovator and early adopter programs • Internal MB stakeholders responsible for integrating CBME data into certification processes	• Identify specific programs for piloting • Revise the outcomes framework as needed based on pilot testing • Develop an approach to data collection and research/quality improvement related to pilot testing • Adapt internal MB processes to prepare for integration of CBME data as needed/appropriate	• As above • Consider how to balance data from GME programs for certification-decision making with independent MB assessments	12-24 mos
Before next stage: Assess pilot testing to understand the multiple, complex educational and system contexts of implementation				
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	• As above	• Communicate outcomes framework as core to training in your specialty • Disseminate tools, assessments, and faculty development materials • Continue data collection and research/quality improvement related to pilot testing • Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	• As above • Creating consistency across programs regardless of local resources available • Culture change needed to change approach to assessment • Learning environment counter to effective CBME implementation • Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
Before next stage: Complete implementation throughout the specialty and develop a strategy for integration of CBME-related data from training into certification processes. Share outcomes widely and improve processes based on outcomes.				
Integrating & Maintaining	• As above	• Adapt CBME processes as needed to continue to improve assessment and decision-making • Continue data collection and research/quality improvement related to pilot testing • Use CBME data for certification decisions	• As above	Ongoing

CBME Shifts the Focus from Time Spent to Outcomes Achieved

- CBME begins with the needs of patients, then works backward to define the outcomes physicians must be able to demonstrate
- Greater emphasis on progression, observation, feedback, coaching, and readiness for unsupervised practice
- For boards, CBME raises a central question:

What evidence should give us confidence that a trainee is ready for unsupervised practice?

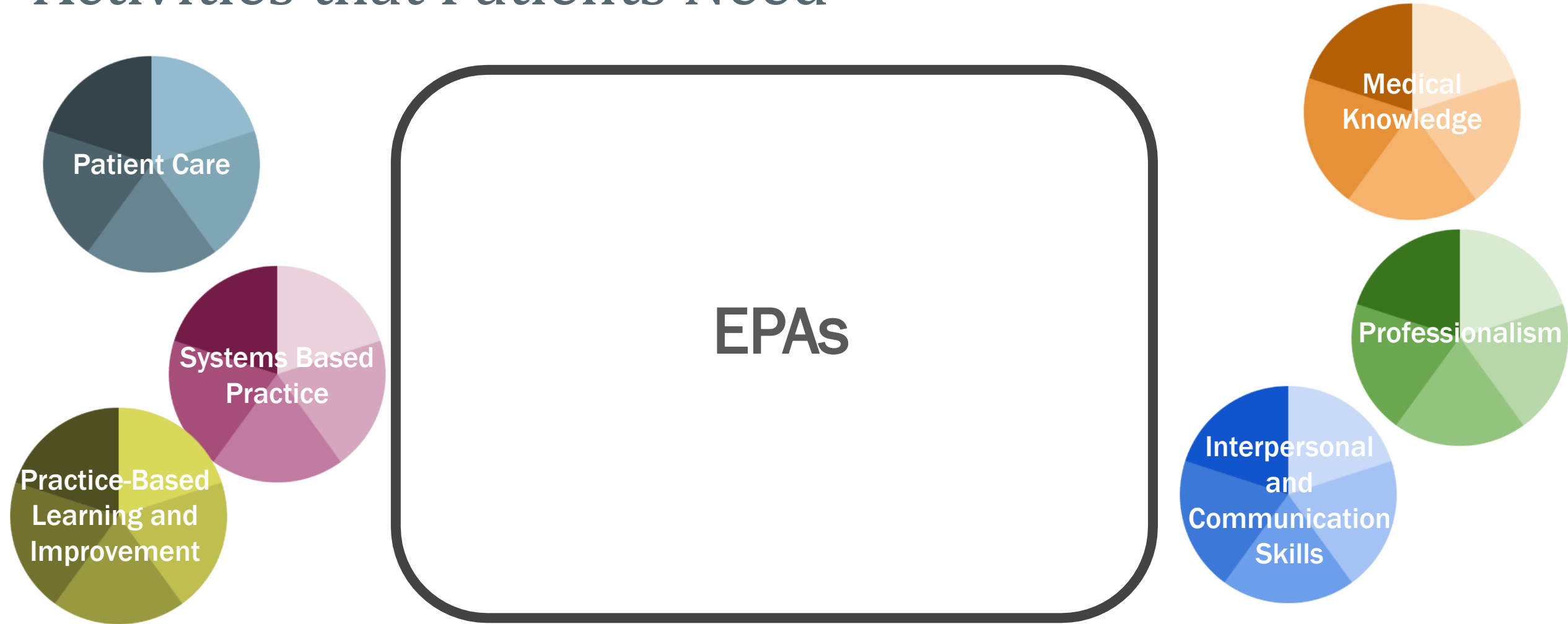
Current Approach to Competency-Based Assessment is Primarily Based on Milestones

Patient Care 5: Patient Management				
Level 1	Level 2	Level 3	Level 4	Level 5
Reports management plans developed by others	Participates in the creation of management plans	Develops an interdisciplinary management plan for common and typical diagnoses	Develops and implements informed management plans for complicated and atypical diagnoses, with the ability to modify plans as necessary	Serves as a role model and coach for development of management plans for complicated and atypical diagnoses, with the ability to modify plans as necessary
☐	☐	☐	☐	☐
Comments:				
Not Yet Completed Level 1 ☐				
Not Yet Assessable ☐				



Milestones Leave Gaps in Assessment and
Create an Incomplete Picture

EPAs Bring Together the Competencies and Focus on Activities that Patients Need





OUR VISION

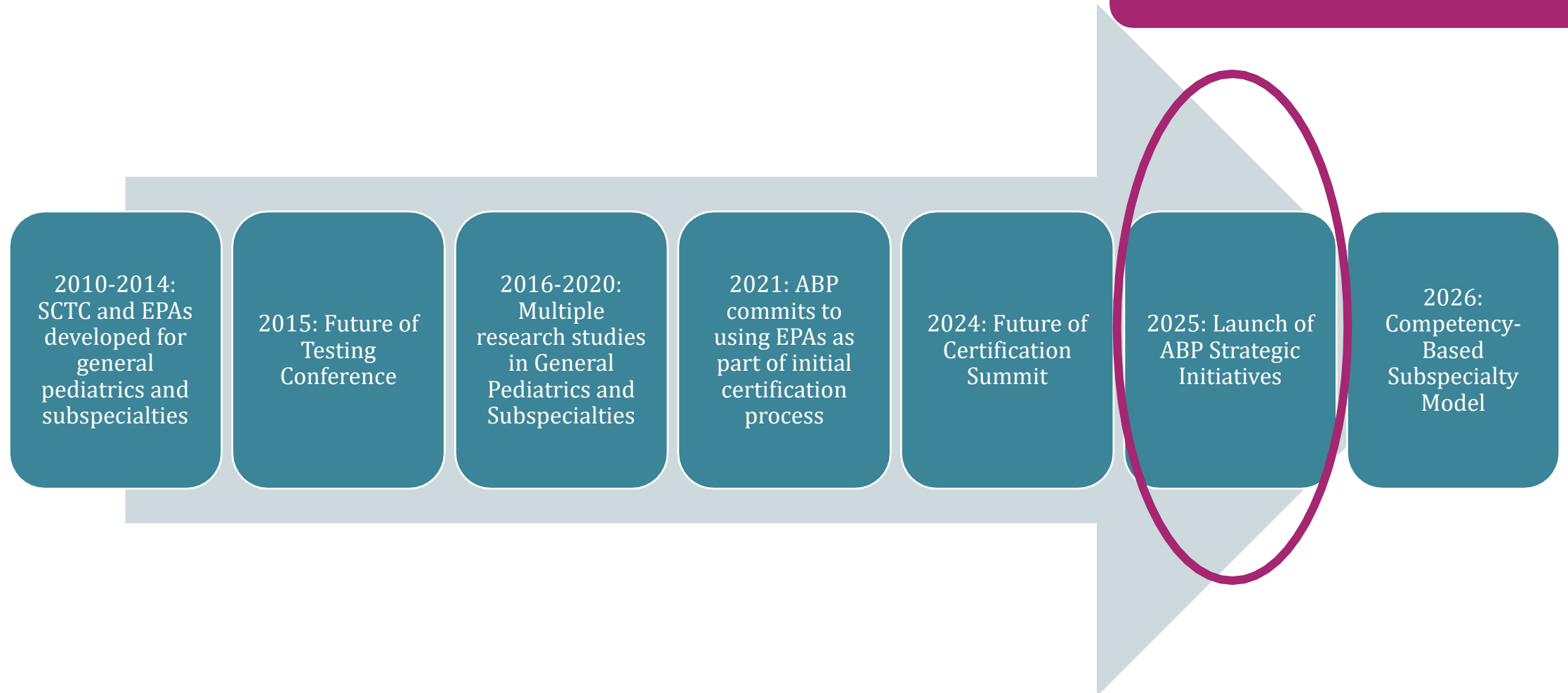
Advancing the
health of all children.



The Pediatrics CBME journey began
many years (decades...) ago

Historical CBME Timeline

Advance CBME in training
and certification

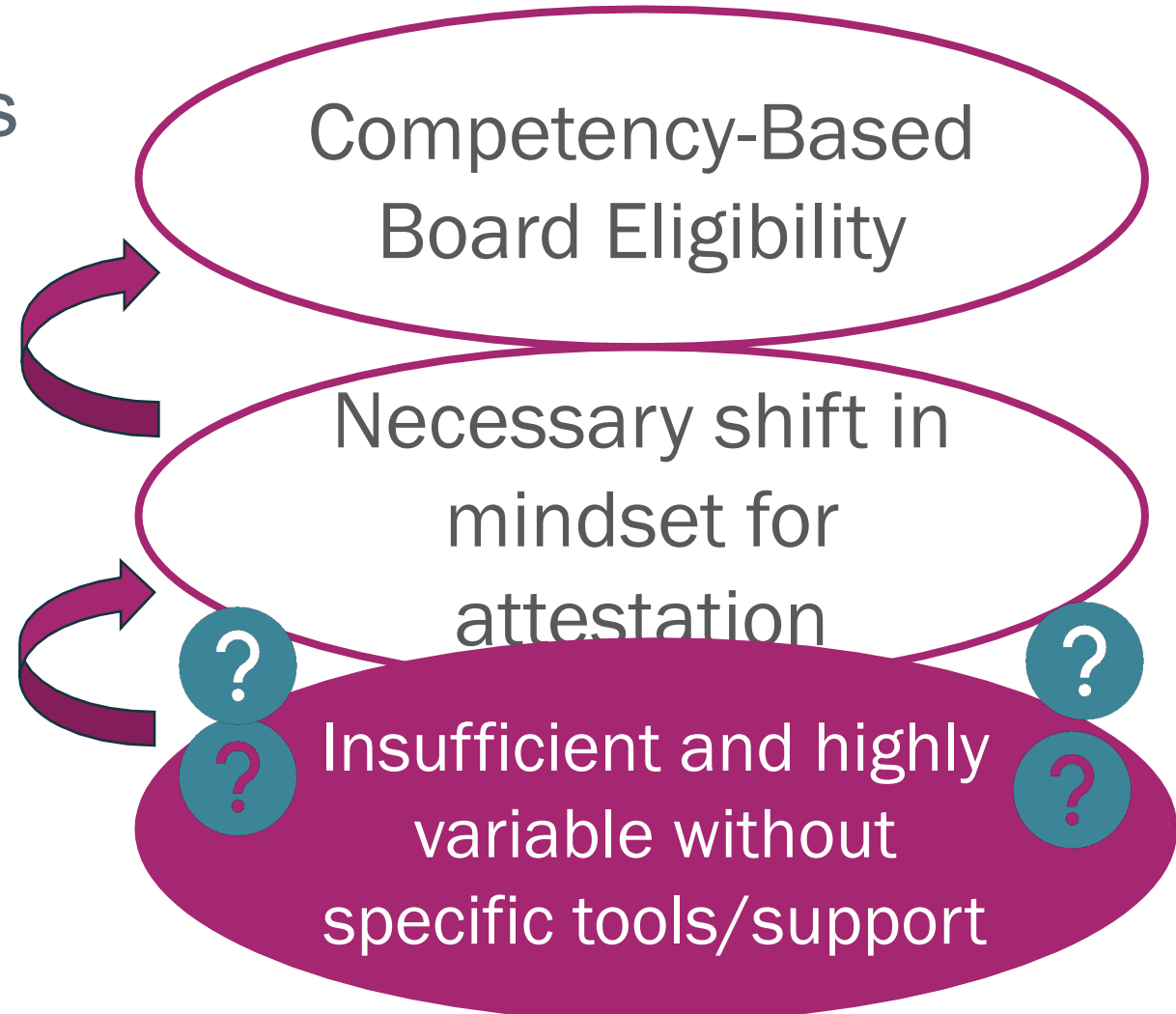


**ABP's Current Work:
Moving From Framework to
Implementation**

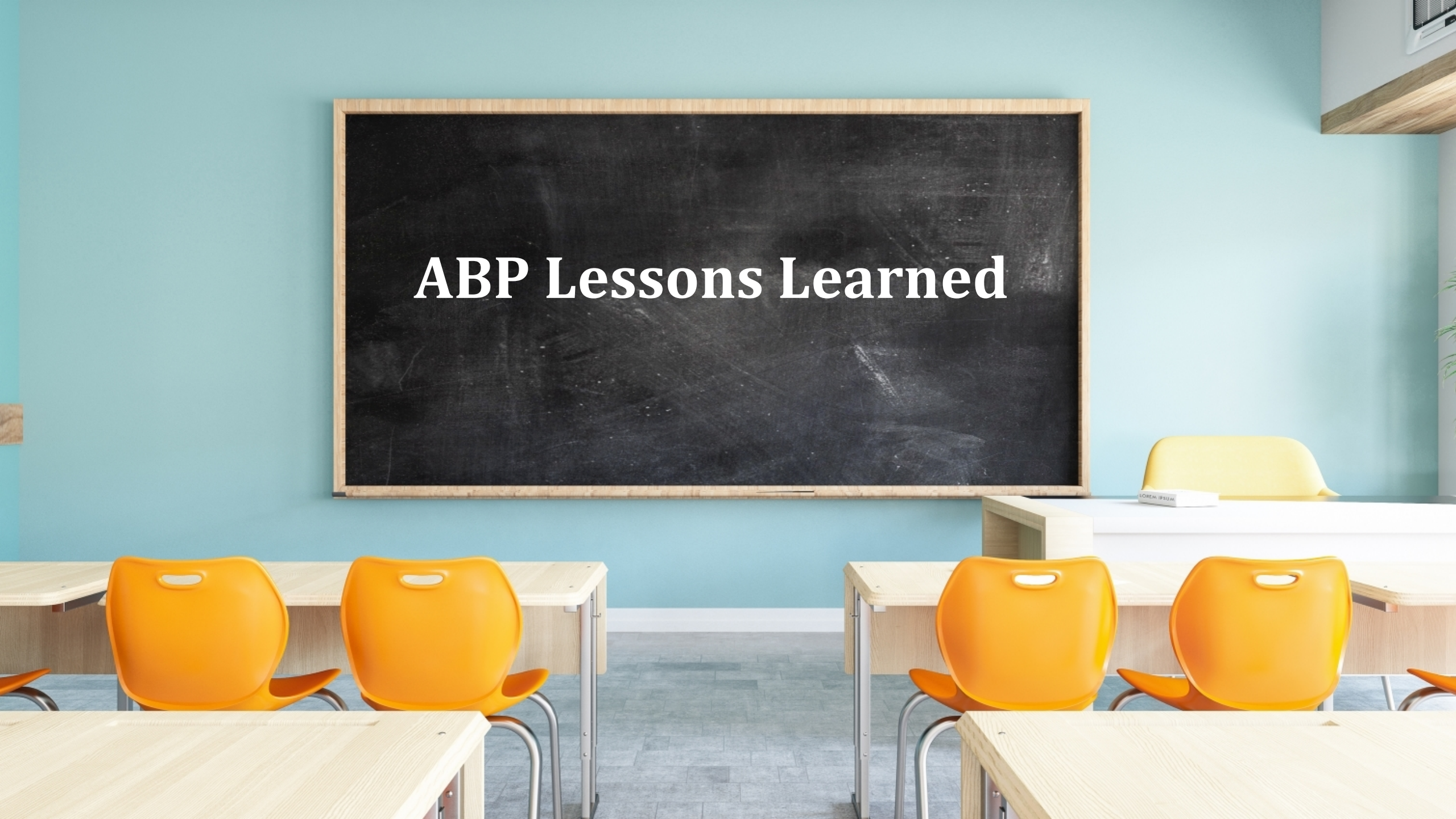
When we say, 'Implement CBME'

What exactly do we mean?

- Program attestation of readiness to practice using the EPA framework (***starts in 2028***)
- Integration of EPAs into Clinical Competency Committee (CCC) decision-making
- Integration of core components of CBME (including EPA-based frontline assessments)



ABP Lessons Learned





Lesson 1: CBME is More Than EPAs

EPAs
represent
the
expected
outcomes
of our
CBME
framework

Lesson 2: National Scaffolding Supports Local Implementation

- Boards can help clarify expectations for readiness
- Programs and faculty are essential to assessment implementation
- Accrediting bodies, specialty societies, program directors, learners, and boards all influence the system
- Sustainable change requires shared language, shared expectations, and practical tools



Lesson 3: CBME Is a Journey, Not a Destination

- CBME implementation is ongoing rather than finite
- Progress occurs through periods of rapid advancement and slower growth
- Pilots, feedback, and course corrections are expected
- Returning to the underlying purpose helps sustain momentum through challenges
- Transformation requires disruption, persistence, and long-term commitment



What This Means Across Member Boards

- How do we define readiness for independent practice in our specialties?
- What evidence should support entrustment decision?
- How can assessment support both learning and certification?
- What should be standardized, and where is flexibility needed?
- Where could Member Boards learn from each other?

The background of the image is a dense, repeating pattern of yellow smiley face emojis (😊). Each emoji is a bright yellow circle with a simple black outline for a face, featuring two dots for eyes and a curved line for a smiling mouth. They are packed closely together, creating a vibrant and cheerful texture.

Thank You!

From Vision to Practice: The American Board of Allergy and Immunology Competency Based Medical Education (CBME) Journey

Michael R. Nelson, MD, PhD
Chair Elect, ABMS
President, ABAI



Allergy and Immunology by the Numbers

- A&I Training Programs
 - 2-3 year fellowship
 - Peds or IM and ABP/ABIM certification prerequisite*
 - 94 ACGME accredited programs
 - 924 faculty (avg 10/prog, 6 core/prog)
 - AY 2025-26
 - 362 total in training
 - 183 starts
- ABAI Diplomates
 - 8000+ certificates
 - 5200+ identify as practicing
 - Outpatient >> Inpatient
 - >60% private practice
 - ~35% academic



**ABAI is a Conjoint Board with ABIM & ABP Sponsors*



American Board of Allergy and Immunology

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- ABP- Jeffrey Stokes, MD (outgoing)
- ABMS 3C- Anna Nowak-Wegrzyn, MD

Test Writing Committee

- **17-18 Members & Ex-officio Chair**

Ambassador Program

- **~60 volunteers**



ABAI Certification

Initial Certification

- ABP or ABIM certification
- Valid unrestricted license
- Candidate application
- Program Director evaluations
- PD competency & procedural skills attestation
- Diplomate recommendation letters

Continuing Certification

- Part I: Professional Standing
- Part II: Lifelong Learning and Self-Assessment
 - CME
 - Pt Safety module/activity q10 yrs
- Part III: Cognitive Expertise
 - Continuous Assessment Program
- Part IV: QI/Improving Health & Health Care
 - Practice Assessment/QI Activities*

What's Missing?!



The Allergy-Immunology CBME/EPA Journey



2023 "Exploring"



2024 Exploring



2025
Developing



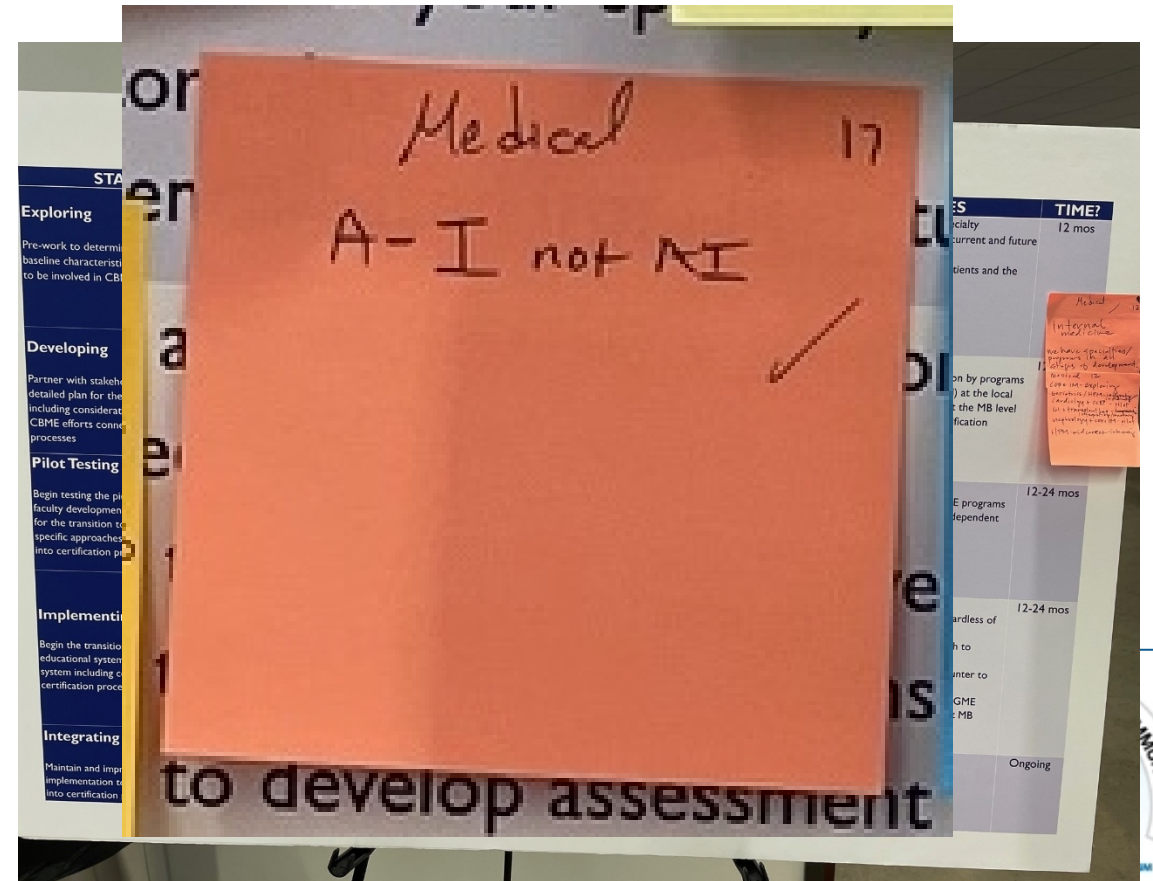
2026 Piloting



2??? Implementing



• 2024



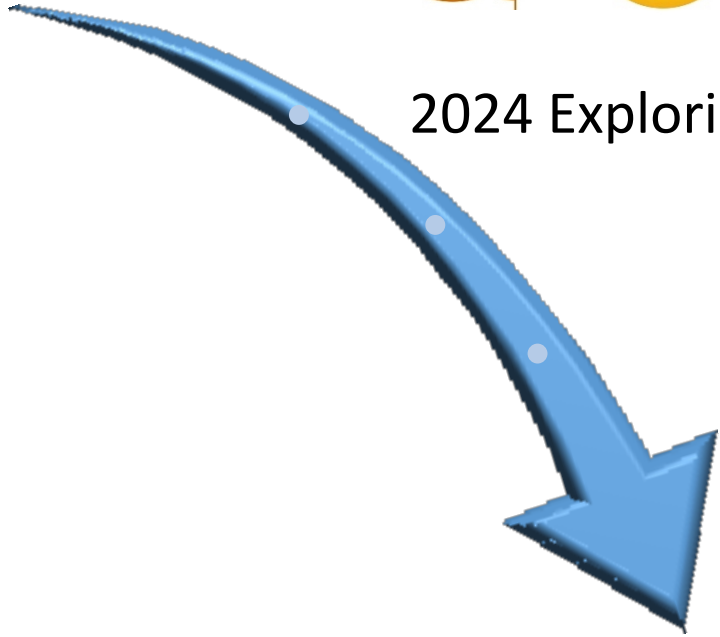


Lessons Learned: Exploring

2023 “Exploring”



2024 Exploring



g

- ABMS/ACGME CBME LC
- Identify & empower champions
- Stakeholder team & roles/resp.
- Identify potential tools
- Create a timeline (on the clock)

Allergy-Immunology CBME Initial Action Plan

- Educate and align key groups (RC-AI, ABAI, PDA, and FITs)
- Cautiously incorporate CBME components into program requirements
- ABAI & A&I RC harmonization of requirements, reporting, documentation, attestations and goals
- “Initiate the development” of A&I EPAs
 - Maintaining “less is more” posture
 - Mindful of burden, cost and scalability
 - Role responsibility TBD?
- Identify and train ambassadors...

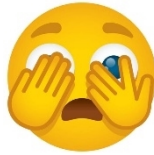
*Do you know
your specialty's
action plan?!*



Lessons Learned: Piloting



2023 “Exploring”



2024 Exploring



2025
Developing



2026 Piloting



- Start small
- Add something, take something away
- Don't be bashful about stealing
- Faculty/learner development
- Co-creation model
- Embed outcomes and metrics
- Create a CBME buzz early
- Regular team & stakeholder meetings
- Share good news early
- Look ahead resource plan
 - ABMS (Scholars, REF grants, toolkit?)

	Milestones	Faculty Min Completion	Fellow minimum per block (3 months) until entrustment (CCC)
Simple/ Uncomplicated	PC1, PC2, PC3, PC4, MK2, SBP2, PBLI1, IPC1	2 per month OR 6 per block	12 immunology, 12 allergy
Complicated/ complex	PC1, PC2, PC3, PC4, MK2, SBP2, PBLI1, IPC1		6 immunology , 6 allergy
Procedures	PC3		2 AIT/VIT, 2 food challenges, 2 drug challenges, 4 PFT, 4 SPT, 2 IgRT
Medical emergencies	PC1, PC2, PC3, PC4, MK2, SPB1, SBP2, SBP3, PBLI1, PRO2, PRO3, IPC1, IPC2, IPC3		2 of any
Communication	SBP2, SBP3, PRO1, PRO2, PRO3, IPC1, IPC2		2 interprofessional; 6 MyChart
Scholarly activities	MK1, MK2, MK3, SBP1, PRO2, PRO3		Scholarly Research Oversight Committee with complete independently
Professionalism	PLBI2, PRO1, PRO2, PRO3,		1 from each attending early and late in block (2)

Lessons Learned: Implementation



2023 “Exploring”



2024 Exploring



2025
Developing



2026 Piloting



2??? Implementing



- ????

- From others

- Phone a friend
- Timeline with all in date
- Resources & funding
- Communicate, listen & adapt
- Technology & data management
- Faculty/learner development
- Use the data
- Study & validate

**Where is your specialty on its
CBME journey?**



Stages of Competency Based Medical Education (CBME) Development within a Specialty

Stage	Collaborators	Actions	Anticipated Challenges	Time?
Exploring Pre-work to determine the specific needs, baseline characteristics, and stakeholders to be involved in CBME planning	- MB representative(s) - ACGME Review Committee leaders - Education leaders within your specialty - Program directors - Experts from other specialties - Patients and families	Convene a working group to explore and identify educational gaps in your specialty/field, readiness for practice of graduates Explore development of clear outcomes in your specialty that are important to patients and families and expected of graduating trainees Review the assessment practices and current time/volume requirements for certification in your specialty (Historical; Evidence from literature; Patient and public needs)	- Resistance to change within the specialty - Data do not readily allow for integration of future processes - Unclear about how to integrate trainees and the public	12 mos
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	Before next stage: Review data regarding readiness to practice of new graduates in the specialty and share this information widely to help define the 'why' for CBME Above, education, and practice gaps for your specialty as described above Form multiple workgroups	Communicate current educational and practice gaps for your specialty as described above Finalize definitions of expected educational outcomes (e.g., entrustable professional activities (EPAs)) Identify faculty development needs and materials Identify technological solutions for implementation Develop or assess the assessment tools for implementation by GME programs, MB, or both Determine how certification process might need to evolve to integrate CBME principles and assessment	Faculty awareness Technological barriers to data collection by programs Resource limitation (people & financial) at the local level to lead/guide CBME efforts and at the MB level to integrate CBME principles into certification processes	12-18 mos
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	- As above - Enlist innovator and early adopter programs - Internal MB stakeholders responsible for integrating CBME data into certification processes	- Identify sites for piloting - Review and develop work as needed based on data collected - Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification) - Adapt internal MB processes to prepare for integration as needed/appropriate	- As above - Consider how to balance data from GME programs with independent certification-decision making with independent MB assessments	12-24 mos
Implementing Begin the transition from current educational system to the new CBME system including collecting CBME data for certification processes	As above	- Communicate outcomes framework as core to training in your specialty - Disseminate tools, assessments, and faculty development materials - Continue data collection and research/quality improvement related to pilot testing - Develop a plan for how CBME data will be used (e.g., advancement, remediation, board certification)	- As above - Creating consistency across programs regardless of local resources available - Cultural approach to assessment - Learning effective CBME implementation - Determine the balance between data from GME programs for certification with independent MB assessments	12-24 mos
Integrating & Maintaining	As above	- Adapt CBME processes as needed to continue to improve assessment and decision-making - Continue data collection and research/quality improvement related to pilot testing - Use CBME data for certification decisions	As above	Ongoing

A&I Next Steps

- Continuing education of our community
- Society & other stakeholder engagement
- EPAs
 - Pilot & PD survey results
 - Refine EPAs
 - Pilot expansion
 - Platform & data management plan
- ABAI & ACGME requirements and attestation alignment
- CBME/EPA/promotion in place institutional barrier exploration

- Maybe someday joining vanguard peers

Phases of Competency Based Medical Education (CBME) Development within a Specialty Graduate Medical Education (GME) and Certifying Member Board (MB)

STAGE	LABORATORS	ACTIONS	CHALLENGES	TIME?
Exploring Pre-work to determine the specific baseline characteristics, and stakeholders to be involved in CBME planning	Identify key stakeholders within Exploring	Working group to explore potential for CBME development	Resistance to change within the specialty Data on educational outcomes for current and future processes not readily available Unclear about expectations from patients and the public	12 mos
Developing Partner with stakeholders to create a detailed plan for the transition to CBME including consideration for how GME CBME efforts connect to certification processes	Identify key stakeholders within Developing	Develop a plan for the transition to CBME	Resistance to change within the specialty Data on educational outcomes for current and future processes not readily available Unclear about expectations from patients and the public	12-24 mos
Pilot Testing Begin testing the pieces (assessment tools, faculty development, technology) needed for the transition to CBME. Develop specific approaches to integrating data into certification processes	Identify key stakeholders within Pilot Testing	Test the pieces (assessment tools, faculty development, technology) needed for the transition to CBME	Resistance to change within the specialty Data on educational outcomes for current and future processes not readily available Unclear about expectations from patients and the public	12-24 mos
Implementing Begin the transition from current educational system to the new CBME system including integrating CBME data for certification processes	Identify key stakeholders within Implementing	Implement the new CBME system	Resistance to change within the specialty Data on educational outcomes for current and future processes not readily available Unclear about expectations from patients and the public	12-24 mos
Integrating & Maintaining Maintain and improve GME implementation to integrate CBME data into certification processes	Identify key stakeholders within Integrating & Maintaining	Maintain and improve GME implementation to integrate CBME data into certification processes	Resistance to change within the specialty Data on educational outcomes for current and future processes not readily available Unclear about expectations from patients and the public	Ongoing

If you've seen one MB journey...

You've seen one MB journey!



Thank you!



ABAI Unanswered Questions



1. How best to engage others in expanding the pilot and for the leap to full implementation?
2. Best communication strategies for engaging stakeholders who want nothing else more to do?
3. Lower number of broad EPAs with forced distribution vs higher number of discrete EPAs?
4. Pearls for EPA faculty development to ensure consistent, actionable and honest use?
5. Advice on a dedicated Professionalism EPA or it's entrustment scale?
6. Institutional policy barriers and strategies to overcome them?
7. Outcome measures/metrics to validate success?
8. ABMS/ACGME Community questions
 - a. Preferred model for primary ownership/oversight of EPA implementation?
 - b. Is ABMS/MBs/ACGME/RCs fully committed to CMBE/EPA? Specialty specific?



CBME, Milestones, Competencies, Milestones & EPAs

- CBME

- Outcomes
- Needs of
- Integrating
- domain
- Direct or
- frequent feedback
- EPAs as one tool

All graduates possess the abilities needed for safe, effective, unsupervised clinical practice, thereby fulfilling medicine's social contract with society



Subcompetency milestones: building blocks

"This resident demonstrates comprehensive knowledge of the varying presentation and alternative and adjuvant treatments"

Communication
and practice

EPAs



EPAs: putting it all together

"I would trust this resident to treat a patient with gallbladder disease with (eg, indirect supervision)."

Montgomery KB et al. JAMA Surg. 2024;159(5):571-577